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SOCIETE GENERALE GROUP

2 November 2026

Price Target Change


Justin Smith
+44 20 7762 5899
justin.smith@bernsteinsg.com


Shan Mian
+44 20 7676 8981
shan.mian@bernsteinsg.com


Maximilian Brewster
+44 20 7550 2193
maximilian.brewster@bernsteinsg.com

Specialist Sales


Christian Moore
+1 917 344 8555
christian.moore@bernsteinsg.com

European Biopharmaceuticals

GSK plc

Rating

Outperform

Price Target

GSK.LN **3,004.00 GBp** (3,010.00 *OLD*)

GSK (O): why's our HIV upside thesis intact?

We still believe that the durability of the HIV franchise is underappreciated due to GSK's entrenched 1st mover advantage in the transition from tablets to long acting injections within the largest segment (treatment = 90% of total US volumes). Hence, our new adj EPSe (26-35e) are upto 28%>consensus (Visible Alpha). We also think '27-'28 pipeline readouts will bring welcome therapeutic diversification ex HIV. Moreover, our PT implies an adj PE30 of 12x which is cheap on global peer group sales regression.

What's our view on the recent HIV pipeline uncertainty? The thrice yearly (CAB Q4M) injection (in treatment and prevention (PrEP)) accounts for 3% of our GSK group DCF (£33.18). We expect VH184 to be the dominant value driver as its more convenient (ie twice yearly which raises patient footfall) and has lower economic payaways. Although CAB is very hard to manufacture, VH184 has enjoys far longer patent life.

What are the other catalysts? In 4Q26 it's the FDA approval of Nuvalent's neldalkib (ALK inhibitor) in second line (2L) non-small cell lung cancer (NSCLC). In '27 its the phase 3 data readout for Nuvalent's Jideytro (ROS1 inhibitor) in 1L NSCLC. In '27 there's also the phase 3 data for 3X yearly injections of Cabenuva (CAB-400) in HIV treatment (90% of the market). In '28, we expect inaugural phase 3 read-outs for three new ex-HIV drugs.

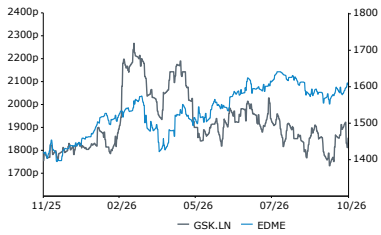
What's changed & why are our new adj EPSe >> consensus? We raise our '26e adj EPS 2% due to margins. We cut our '27-32e adj EPS by 1-3% due softer Vaccines (Arexy; prevents RSV) and General Medicines (US deflation)). We increase our '35-36e adj EPS by 2-3% due to the pipeline (oncology). Our adj EPS CAGR (26-31e) drops by 90bp's to 7.5%, which is in-line with versus EU peers on Bernstein estimates. (Our adj EPS CAGR (26-36e) remains 6.8%). Our new '26-35e adj EPS are upto 28% >consensus (Visible Alpha throughout) due to sales and margins. (We assume 75% of the £1.9bn 'accelerate growth' savings program is reinvested).

Close Date	29 Oct 2026
GSK.LN Close Price (GBp)	1,814.00
Price Target (GBp)	3,004.00
Upside/(Downside)	66%
52-Week Range	2,282.00/1,722.50
EDME	1,603.66
FYE	Dec
Div Yield	3.8%
Market Cap (GBP) (M)	74,436
EV (GBp) (M)	94,783

Performance	YTD	1M	6M	12M
Absolute (%)	0.9	0.4	(4.6)	3.2
EDME (%)	9.1	2.4	5.9	12.9
Relative (%)	(8.2)	(1.9)	(10.4)	(9.8)

Source: Bloomberg, Bernstein estimates and analysis.

Price Performance, 1YR



Investment Implications

How do we calculate our Price Target (PT)? Still via the average of **(i)** DCF (£33.18 up 5%; WACC and terminal growth remain 8% and 1% **(ii)** '27-'29e EV/ EBITA (£26.90 down 6%). Our PT drops 6p to £30.04 as our long term upgrades are offset by peer group derating and our ST/MT downgrades. Our new PT implies an adj PE30e of 12x for 6% premium to EU peers. On global large cap pharma 5-year EV/sales regression GSK is cheap at our new PT.

Adjusted EPS	F25A	F26E	F27E
GSK.LN (GBP)	1.69	**1.82**	**2.00**
OLD	--	1.79	2.06

Financials	F25A	F26E	F27E	CAGR
Revenues (M)	32,667	33,828	36,179	--

Valuation Metrics	F25A	F26E	F27E
Adjusted P/E (x)	10.7	10.0	9.1

Source: Bloomberg, Bernstein estimates and analysis.

See the Disclosure Appendix of this report for required disclosures, analyst certifications and other important information. Alternatively, visit our Global Research Disclosure Website.

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DETAILS

WHAT'S CHANGED?

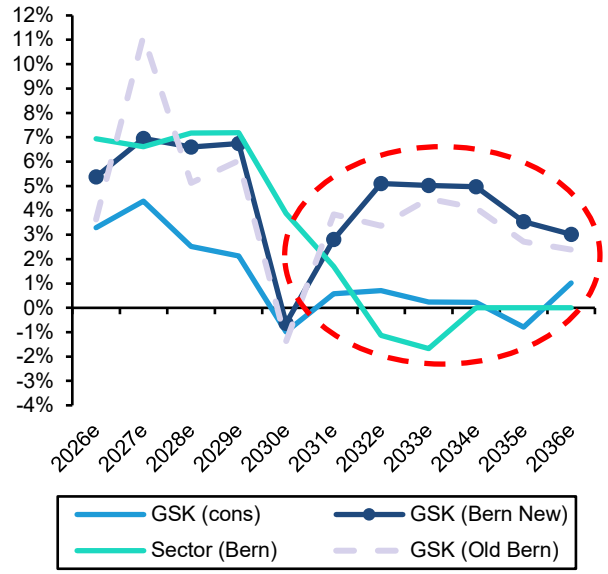
EXHIBIT 1: **Bernstein GSK group core EPS forecast changes**

GSK group core EPS	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E	2035E	2036E	CAGR 26-31E	CAGR 26-36E
Bernstein old	178.7	205.8	224.4	245.4	250.9	268.5	287.6	305.8	325.0	336.3	343.8	8.5%	6.8%
Bern old Growth	*5%*	*15%*	*9%*	*9%*	*2%*	*7%*	*7%*	*6%*	*6%*	*3%*	*2%*		
Bernstein new	182.3	199.9	218.0	240.3	247.8	262.2	286.0	305.8	328.1	342.3	352.3	7.5%	6.8%
Bern new Growth	*8%*	*10%*	*9%*	*10%*	*3%*	*6%*	*9%*	*7%*	*7%*	*4%*	*3%*		
% change	2.0%	-2.9%	-2.8%	-2.1%	-1.2%	-2.3%	-0.6%	0.0%	1.0%	1.8%	2.5%	-93bps	5bps
Bloomberg Consensus	179.9	187.8	197.6	209.0	212.1	227.0	239.7	257.0	300.4	344.6	473.4	4.8%	10.2%
Bloomberg Consensus Growth		*4%*	*5%*	*6%*	*1%*	*7%*	*6%*	*7%*	*17%*	*15%*	*37%*		
% difference: Bern new vs cons	1%	6%	10%	15%	17%	16%	19%	19%	9%	-1%			
VA Consensus	176.4	186.6	193.4	203.1	206.2	215.3	224.8	238.0	275.4	328.8	702.4	4.1%	14.8%
VA Consensus Growth		*6%*	*4%*	*5%*	*2%*	*4%*	*4%*	*6%*	*16%*	*19%*	*114%*		
% difference: Bern new vs cons	3%	7%	13%	18%	20%	22%	27%	28%	19%	4%			

Source: Company reports, Bernstein analysis and estimates, Bloomberg, Visible Alpha

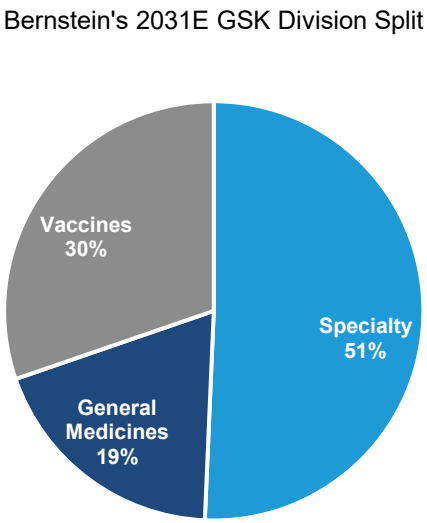
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EXHIBIT 2: **GSK group sales: Bernstein old vs new vs EU Pharma vs consensus**



EU Pharma sector: AZN, GSK, NOVN, ROP, SAN, NOVOB, MRK GY, GMAB, ARGX
Source: Company reports, Bernstein analysis and estimates, Visible Alpha

EXHIBIT 3: **Bernstein's 2031E GSK Division Split**



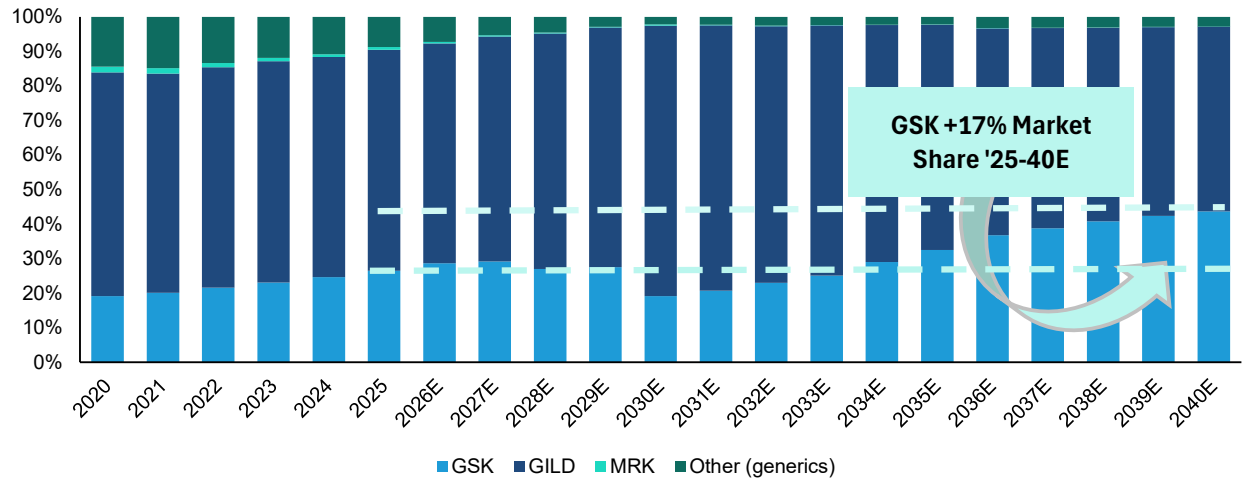
Source: Company reports, Bernstein analysis and estimates

WHY'S OUR HIV UPSIDE THESIS STILL INTACT?

In June 2026, we built a new "deep-dive" proprietary HIV model to 2040, which took weeks to fine tune (the accounting is very tricky). We included forecasts for GSK's novel long acting (LA) drugs which we believe offer the combination of greater convenience and (in the case of VH184) far longer patent protection. We thus now believe that this enhanced portfolio will better position GSK to gently, but sustainably, gain share upto 17 points of share in the US HIV market (Exhibit 4) from its larger rival Gilead (O; covered by Courtney Breen) by 2040.

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EXHIBIT 4: **We believe that GSK is well positioned to capture 17 points of US HIV market share by 2040e**



Source: Company reports, Bernstein analysis and estimates

GSK's current FDA approved long acting (LA) HIV portfolio currently comprises Cabenuva (for Treatment) and Apretude (for prevention; PrEP) intramuscular injections every other month (Q2M; Exhibit 6**).** Both Cabenuva and Apretude contain the second generation integrase inhibitor (INSTI) cabotegravir which is expected to suffer US loss of exclusivity in 2031e. A "higher-dose" formulation (which enjoys half dosing interval (Q4M)) of cabotegravir is currently undergoing late-stage clinical trials for both Treatment and PrEP.

We are still expecting the phase IIb (EXTEND 4M) data read out for GSK's cabotegravir 4QM (3X yearly injection) in HIV (prevention) PrEP (i.e. a higher dose of Apretude) in 2H26, and a US launch in 2H27 (Exhibit 6). The results of this pharmacokinetic (PK) study (which examines how the body absorbs the drug after administration) are now in house. We also understand that GSK is analyzing these data and to fully characterize the new formulation before sharing these data with the regulators pre 31/12/26.

GSK has also clearly indicated that EXTEND 4M hasn't identified any safety or efficacy issues. Management has also emphasised that there is no read across from EXTEND 4M to the phase III CUATRO trial for a higher dose of Cabenuva (cabotegravir 4QM + rilpivirine) for HIV Treatment, which was initiated in 2Q26 (Exhibit 6) and is now fully recruited.

We also believe that the launches of higher dose (Q4M) cabotegravir will make generic competition (the substance patent expires in 2031) to this molecule much more challenging because the manufacturing process for the Q2M version is already extremely difficult (at the 2Q26 "accelerate growth event" in July, we learned that cabotegravir is the most difficult drug in the GSK portfolio to manufacture). The FDA delayed the original approval of Cabenuva Q2M by 12 months for manufacturing reasons.

We also believe launching these new HIV drugs will accelerate the well established shift of patients onto long acting injectables (LAs), away from daily oral tablets, prior to the launch of GSK's third-generation integrase inhibitor (INSTI; VH184, administered twice per year) which we expect to debut in 2031.

But we believe the long term value for GSK's HIV franchise will be driven by its novel (twice yearly) Integrase Inhibitor VH184. VH184 will be now developed in HIV Treatment, and has substance of matter patent protection until 2040s. Thus, we believe that VH184 is a vital tool to extend GSK's HIV franchise beyond the loss of exclusivity (LOE) of cabotegravir in 2031 (even if we think the impact of generic entrants to this latter molecule will be benign). GSK has decided for its phase 3 trial in HIV treatment to pair VH184 with a bNAb (broadly neutralizing antibody) or a Capsid Inhibitor (Exhibit 5).

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EXHIBIT 5: **GSK's HIV pipeline**



Source: GSK

Our channel checks also indicate that, because VH184 only has a royalty payment to Shionogi (i.e., unlike Cabenuva (and the legacy HIV tablets containing 1G dolutegravir), there are no contingent liability payments). Hence, 'pound for pound' the sales of VH184 will create more value for GSK's shareholders than all its other HIV medicines which contain an Integrase inhibitor (ie dolutegravir or cabotegravir). This is particularly important because we believe there will be a degree of margin contraction in the late 2020s as Cabenuva accounts for a higher percentage of GSK's total HIV sales (after dolutegravir (DTG) products (Tivicay, Triumeq, Juluca, and Dovato) start to lose exclusivity in 2028. This is because GSK has to pay a very large royalty on the sales (we estimate in the 40s percentage) of Cabenuva because J&J (M; Lee Hambright) invented one of the two components, rilpivirine.

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EXHIBIT 6: **GSK HIV franchise overview**

	Portfolio						Pipeline		
Formulation	Oral				Injectable				
Regimen	Daily				6X Yearly		3X Yearly		2X Yearly
Market	Treatment					PrEP	Treatment	PrEP	Treatment
INSTI Backbone	dolutegravir (DTG)				cabotegravir (CAB)				VH184
INSTI Dosage	*50mg*				*600mg*	*600mg*	*Undisclosed*	*Undisclosed*	*Undisclosed*
NRTI		abacavri (ABC), lamivudine (3TC)							
NNRTI			rilpivirine (RPV)	lamivudine (3TC)	rilpivirine (RPV)		rilpivirine (RPV)		
Other Dosage		*600/300mg*	*25mg*	*300mg*	*900mg*				
Branded Name	Tivicay	Triumeq	Juluca	Dovato	Cabenuva	Apretude	CAB-4QM (Treatment)	CAB-4QM (PrEP)	VH184
CCL	"High teens" CCL to Shionogi								
Royalty/ Payaway			30% (BernE) to JNJ		30% (BernE) to JNJ				17.5% (BernE) to Shionogi
Launch	2013	2014	2017	2019	2021	2022	2028E	2027E	2031E
2025 Global Sales (£mn)	1,323	991	656	2,678	1,402	439			
% 2025 HIV sales	17%	13%	9%	35%	18%	6%			
Peak (Actual; £mn)	1,662 (2019)	2,648 (2018)	685 (2024)	2,678 (2025)	1,402 (2025)	439 (2025)			
Bern Peak (£mn)				3,286 (2028E)	2,505 (2027E)	548 (2027E)	2,562 (2031E)	2,282 (2040E)	5,066 (2040E)
VA Cons Peak (£mn)				3,229 (2027E)	2,702 (2030E)	806 (2030E)	700 (2033E)	897 (2040E)	389 (2037E)

Source: Company reports, Bernstein analysis and estimates, Visible Alpha

Justin Smith +44 20 7762 5899 justin.smith@bernsteinsg.com

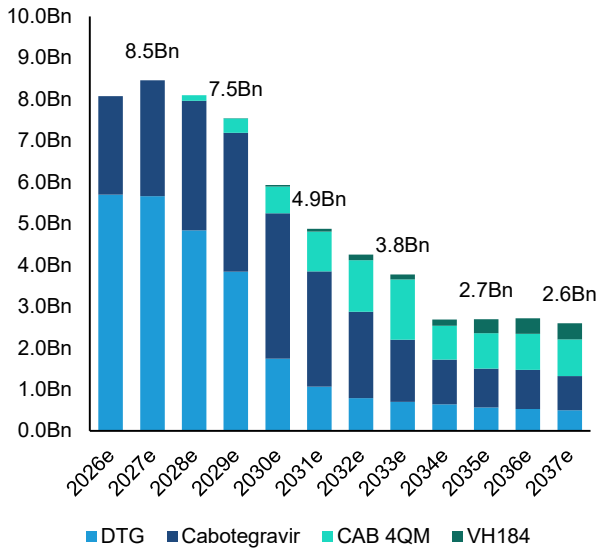
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We assume an 80% probability of success for cabotegravir Q4M (in phase III) as this molecule is already FDA approved as it's contained in Cabenuva and Apretude). We also assume a 40% probability of launch for VH184 (late phase II asset as its mechanism of action is extremely well validated as it's also an Integrase inhibitor like dolutegravir and cabotegravir). We also model a more benign erosion of the sales of "lower-dose" (Q2M) Cabenuva and Apretude. This is because our channel checks have indicated that patient demand for a dosing every other month will be somewhat sticky, even as longer-acting injectables reach the market.

We understand from GSK that the manufacturing process for Cabenuva is extremely complicated and explains why FDA delayed the approval of this product by 12months. During is 07/26 CMD GSK also indicated that it intends to defend US cabotegravir patents (expiring around 2040 at the latest).

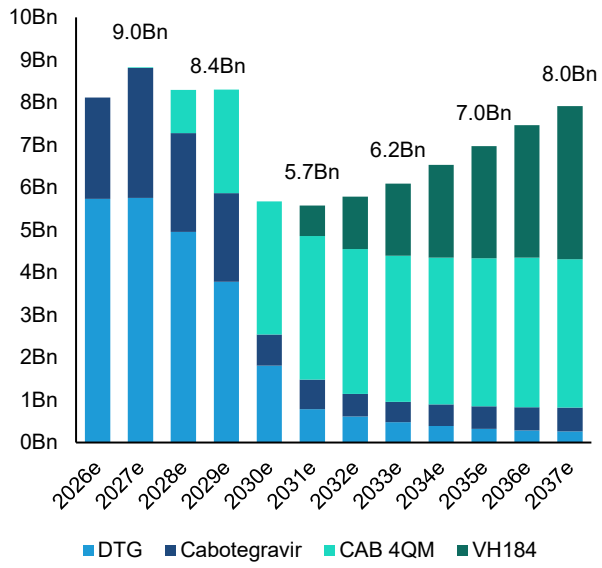
Dolutegravir (DTG; active pharmaceutical ingredient in Tivicay, Triumeq, Dovato, and Juluca daily oral tablets) is set to lose exclusivity in the US in 2029-2030 and started to lose patent protection in major ex. US markets (such as Brazil) from 2026. Our 2036e forecasts for the DTG franchise drugs are largely unchanged. This is because we think GSK will execute well on transitioning patients onto its long acting (LA HIV) drugs prior to the DTG patent expiry in the US.

EXHIBIT 7: **Consensus forecasts on GSK's HIV franchise (£)**



Dosing: DTG (daily tablet); cabotegravir (injection every other month); CAB4M (thrice/year); VH184 (twice/year)
Source: Visible Alpha, Bernstein analysis

EXHIBIT 8: **Bernstein's latest GSK HIV forecasts**



Risk adjustments: CAB-4QM = 80%; VH184 = 40%. Dosing: DTG (daily tablet); cabotegravir (injection every other month); CAB4M (thrice/year); VH184 (twice/year)
Source: Company reports, Bernstein analysis and estimates

GSK was the first player in both HIV Treatment and HIV PrEP to launch a long acting injectable drug. This is because Cabenuva (for treatment) and Apretude (for prevention; PrEP) were launched in the US in 2021 and 2022 respectively. Meanwhile, Gilead (which commanded 64% share of the total US HIV market in 2025) launched twice-yearly Sunleca (Treatment) and Yeztugo (PrEP) in 2023 and 2025 respectively. Both Sunleca and Yeztugo contain the novel capsid inhibitor lenacapavir.

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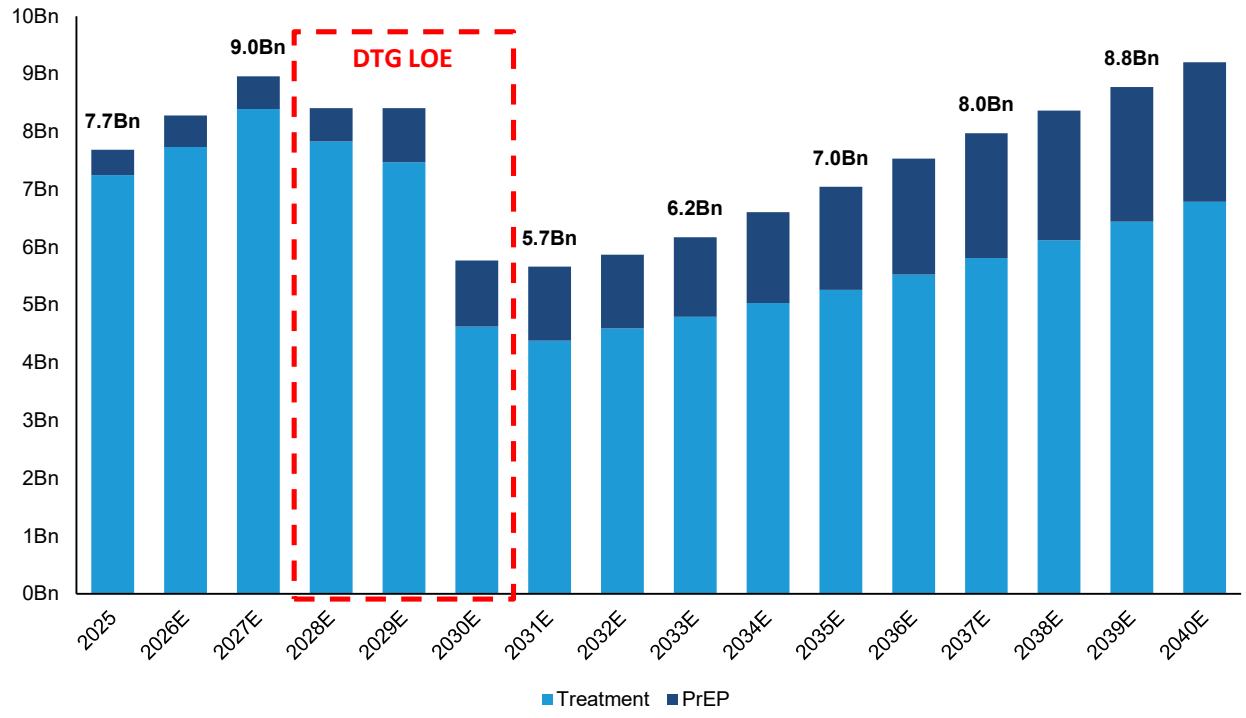
In our view, the potential for Sunleca to take significant share within the Treatment market is limited. This is because we believe that the highly authoritative US National Institute of Health (NIH) guidelines are likely to continue to recommended that patients begin treatment on an ARV (antiretrovial) regimen which consists of an oral second generation INSTI (integrase inhibitor) + two NRTIs (Exhibit 9).

EXHIBIT 9: **US NIH guidelines on HIV Treatment**

Recommended Initial Regimens for Most People with HIV Demonstrated durable virologic efficacy, favorable tolerability and toxicity profiles, and ease of use.
INSTI + 2 NRTIs
Bictegravir/tenofovir alafenamide (TAF)/emtricitabine (FTC) (*AI*)[1]
Dolutegravir/abacavir/lamivudine - **Only if HLA-B*5701 negative and without hepatitis B virus (HBV) coinfection** (*AI*)[1]
Dolutegravir + tenofovir[2]/emtricitabine[3] (*AI*)[1]
Raltegravir[4] + tenofovir[2]/emtricitabine[3] (*BI* for TDF, *BII* for TAF)[1]
INSTI + 1 NRTI
Dolutegravir/lamivudine (AI)[1] **- If HIV RNA < 500,000, no HBV coinfection, and genotype results showing no reverse transcriptase resistance**

Source: HIVinfo.NIH.gov

EXHIBIT 10: **We estimate that HIV prevention (PrEP) will account for 27% of GSK's global HIV revenues in 2040 (£)**



Source: Company reports, Bernstein analysis and estimates

We believe that GSK's first mover advantage in the long acting HIV market will strengthen through to the late 2030s, with Gilead's behemoth Biktarvy (daily oral tablet; 45% of the US HIV market in 2025) losing US patent protection in 2036e.

At present, we cannot find anything meaningful in the medium term Gilead pipeline which we believe can meaningfully compete with GSK's long acting injectables in HIV Treatment. Our channel checks also indicate that HIV patients are a highly motivated patient population, and there is excellent demand for long acting injectables. We believe that these drugs materially improve patient well-being not just because of convenience but also because LA's remove the traumatic daily reminder that forgetting to take pills increases the risk of AIDS and eventually death. Nonetheless, we still forecast Biktarvy to remain the largest drug (by

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sales) in the US HIV Treatment market until its loss of exclusivity (LOE; Exhibit 11) in 2036.

EXHIBIT 11: **Overview of HIV Treatment (top) and PrEP (prevention; bottom) pipeline assets**

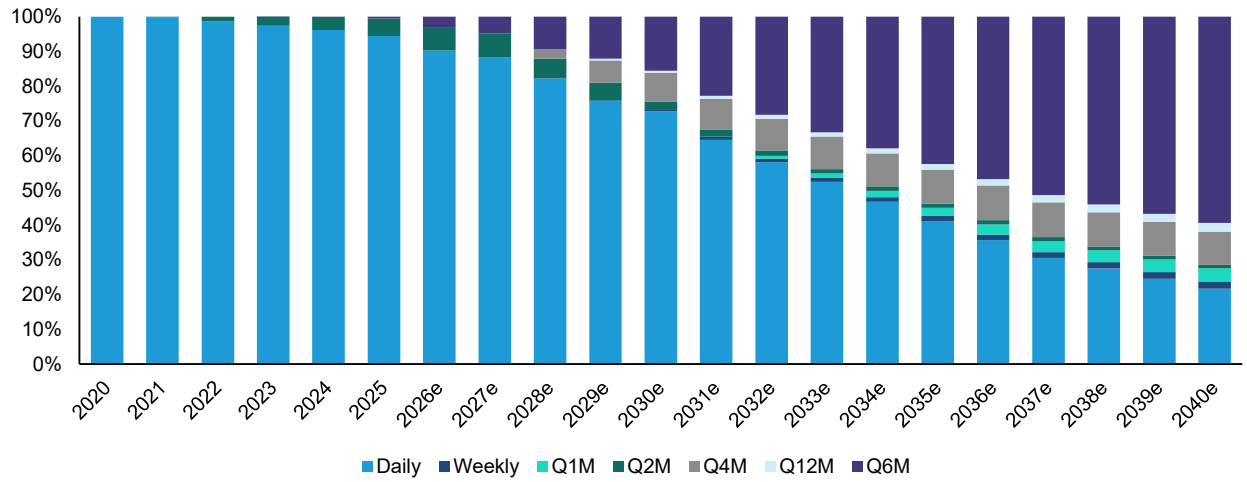
Treatment Product	Ingredients	Phase	Modality	Dosing	Regimen	Est. Launch	Integrase Inhibitor	NRTI	NNRTI	CI	bNAb
GSK											
CAB 400	1	II	small mol.	Q4M	Injectable	2027 (Q4M)	cabotegravir (CAB)				
VH184	1	II	small mol.	Q6M	Injectable	2030	VH184				
+VH109	2	II	bNAb	N/A		2033	VH184			VH109	
+VH499	2	II	small mol.	N/A		2033	VH184				VH499
GILD											
Bic + Len	2		small mol.	Daily	Oral	2H26	bictegravir (BIC)			lenacapavir (LEN)	
Len + Isla (w/MRK)	2	III	small mol.	Weekly	Oral	2027		islatravir (ISL)		lenacapavir (LEN)	
GS-1720 + GS-4182	2	Clinical Hold	small mol.	Weekly	Oral	2030	GS-1720			GS-4182	
GS-3107 + INSTI	2	I	small mol.	Monthly	Oral	2032	GS-3242			GS-3107	
Len + bNAbs	3	II	bNAb	Q6M	Injectable	2030				lenacapavir (LEN)	teropavimab (TAB), zinlirvimab (ZAB)
Len + GS-3242	1	I	small mol.	Q6M	Injectable	2032	GS-3242			lenacapavir (LEN)	
MRK											
MK-8591B	2	III	small mol.	Weekly	Oral	2027		islatravir (ISL)	ulonivirine (ULO)		

PrEP Product	Ingredients	Phase	Modality	Dosing	Regimen	Est. Launch	Integrase Inhibitor	NRTI	NNRTI	CI	bNAb
GSK											
CAB 400	1	II	small mol.	Q4M	Injectable	2027 (Q4M)	cabotegravir (CAB)				
GILD											
Lenacapavir 12QM	1	III	small mol.	Q12M	Injectable	2028				lenacapavir (LEN)	
MRK											
MK-8527	1	III	small mol.	Monthly	Oral	2027			MK-8527		

Source: Company reports, Bernstein analysis and estimates

Crucially, longer dosing intervals for HIV medications also allows HIV practitioners to increase 'footfall' within their clinics. Our channel checks indicate that most HIV patients will see their doctor 2-3 times a year for routine check-ups. Thus, we believe that perfectly matching drug dosing intervals with patients' visits to their practitioners will allow doctors to see more patients per year. Long Acting HIV drugs should thus offer far better value propositions to healthcare systems than HIV tablets. Furthermore, we understand that HIV patients are highly motivated and that there is latent demand for LAs. Thus, we believe our estimated share splits between dosing regimens are robust (Exhibit 12).

EXHIBIT 12: **Shares of dosing regimens (patients) in our US HIV model**



Q1M = One monthly dosing; Q2M = every other month dosing; Q4M = Four-monhtly dosing; Q6M = Six-monthly dosing; Q12M = Twelve-monthly dosing
Source: IQVIA, Bernstein analysis and estimates

WHY IS THE INFLATION REDUCTION ACT NOT A MATERIAL HEADWIND FOR GSK IN HIV?

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The Inflation Reduction Act's drug pricing program passed into law in 2022. To recap, the Act allows the US government to negotiate prices for certain high-spend Medicare drugs. For the first two years of the program (2026 and 2027) only Medicare Part D drugs (i.e. self administered drugs) are covered, whilst in 2028 and 2029 Medicare Part B (drugs administered in clinical settings by physicians) will also be eligible for US price negotiations. Small-molecule drugs are only eligible for negotiation 9 years post FDA approval, whereas biologics are eligible for negotiation from 13 years (negotiated prices then take effect 2 years after). Across 2027, 2028, and 2029, 15, 15, and 20 drugs will be selected for the program respectively.

On Gilead's (O; covered by Breen) 3Q26 results call, we heard investor concerns about the IRA impact for Biktarvy's (daily oral tablet in HIV Treatment). In 2024 (the relevant year for 2026 price negotiation and 2028 impact), Biktarvy captured $10.9bn of sales in the US (and we understand ~20% was derived via the Medicare channel) and thus was eligible for IRA negotiations.

We believe there is no read across to GSK's HIV franchise for the following reasons:

1. **Price declines are priced into the legacy portfolio:** We forecast GSK's current biggest franchise (dolutegravir (DTG) daily oral tablets) which includes Tivicay, Triumeq, Juluca, and Dovato will lose US exclusivity across 2028-30. Thus we already model material sales declines yoy.

2. **The scope of the IRA is too narrow for short term impact on GSK's long acting portfolio in our view.** GSK's current long acting portfolio uses a cabotegravir backbone (in Cabenuva (6 times yearly Treatment) and Apretude (6 times yearly PrEP)) . This small molecule is expected to be approved in 4 times yearly Treatment and PrEP (CAB Q4M). As long acting injectables are administered in clinical settings (Medicare Part B), cabotegravir would technically only be in the scope of the IRA from 2028. However, considering this molecule was only approved by the FDA in 2021, price negotiations are only eligible in 2030 to be implemented in 2032.

3. **Cabotegravir Medicare sales exposure will be relatively low.** We assume risk-adjusted "total cabotegravir" (i.e. 6 times/ year + 4 times/ year in Treatment and PrEP) sales of $3.2bn in 2030 (earliest negotiation year). Assuming 20% sales exposure to Medicare, we calculate that $633m of sales are exposed to IRA negotiation. We thus believe cabotegravir will not be included in the IRA list of drugs by 2030.

4. **We believe GSK's HIV pipeline assets are attractive and patients will want to switch.** From 2031, cabotegravir loses patent protection in the US. In the same year, we model GSK's next generation Integrase Inhibitor VH184 (2 times yearly Treatment) will launch. We believe that patients will be extremely motivated to switch onto the longer-acting VH184. Thus we forecast Treatment-related cabotegravir sales (64% of GSK's HIV Treatment sales in 2031) will rapidly decline yoy (to 37% of GSK Treatment sales in 2036) as patients opt for twice yearly VH184.

VALUATION

We still calculate our price target (PT) via a straight average of 2027-2029e EV/EBITA multiples and a DCF. In the latter, we apply fade rates from 2041 until 2045. We thus only apply a perpetuity growth rate of 1% for an exit multiple of 14.4x) in 2045. This means that the exit value accounts for 29% of the total EV.

Our long term upgrades have increased our DCF (now £33.18; up 5%). However, we cut our '27-29e EV/EBITA multiples valuation (now £26.9; down 6%) due to softer sales from Vaccines (Arexvy, prevents RSV) and General Medicines (US deflation), as well as peer group multiple contraction. We maintain our assumption of GSK trading at parity-to- EU peers in our '27-29e EV/EBITA calculation.

Taken together these changes now yield a new PT (price target) of £30.04 (down 0.2%; Exhibit 13). Our new PT implies an adj PE30e of 12x for 6% premium to EU peers. On global large cap pharma 5-year EV/sales regression GSK is cheap at our new PT (Exhibit 16).

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EXHIBIT 13: **GSK Price Target Calculation**

GSK - Price Target change (£)	Old	New	% change	Assumptions
DCF	31.57	33.18	5%	WACC remains 8%; perpetuity growth rate increased to 1% from 0%
EV/EBITA (2027-2029e)	28.63	26.90	-6%	5% premium to EU pharma peers cut to parity given NUVL dilution
Average	**30.10**	**30.04**	0%	
Zantac litigation impact				
Target Price (£)	**30.10**	**30.04**	0%	
GSK implied core PE27	14.6x	15.0x		
European Pharma core PE27	14.3x	14.3x		
Implied relative valuation vs EU peers	8%	5%		

Source: Company reports, Bernstein analysis and estimates

Our new PT implies a '27e PE multiple for GSK of 14.3X (from 14.6X), for a 5% premium to EU peers (Exhibit 14). Reverse engineering our new GSK DCF implies that the current share price discounts negative Free Cash Flow growth from 2041 into perpetuity. In our view this extreme considering that GSK now benefits from its most "pro-innovation" CEO for 20 years and is the global leader in vaccines (which in our view enjoy the deepest moats in all healthcare).

EXHIBIT 14: **GSK EV/EBITA Multiples Calculation (£)**

	2027e	2028e	2029e	Premium	Average
EBITA	11,454	12,274	13,085		
Sector multiples (EU pharma inc Novo)	12.3x	11.4x	10.6x	0%	
Applied multiples	12.3x	11.4x	10.6x		
EV	**141,101**	**139,547**	**138,636**		**139,761**
Net debt					22,436
Provisions					2,692
Contingent consideration (HIV)					6,733
Minorities					-421
Market capitalisation					**108,321**
Diluted Shares (m)					4,027
Per share price (£)					**26.90**

Source: Company reports, Bernstein analysis and estimates

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Justin Smith +44 20 7762 5899 justin.smith@bernsteinsg.com

EXHIBIT 15: **GSK Discounted Cash Flow (DCF) valuation (£m unless otherwise stated)**

	2026e	2027e	2028e	2029e	2030e	2031e	2032e	2033e	2034e	2035e	2040e	2045e	2046e	2047e
Sales	33,828	36,179	38,566	41,167	40,907	42,055	44,201	46,424	48,734	50,456	56,937	59,842	60,440	61,044
% growth	4%	7%	7%	7%	-1%	3%	5%	5%	5%	4%	2%	1%	1%	1%
EBITDA	**12,053**	**11,073**	**12,438**	**13,469**	**14,136**	**14,805**	**15,978**	**16,907**	**17,970**	**18,746**	**21,154**	**22,233**	**22,455**	**22,680**
Margin	*36%*	*31%*	*32%*	*33%*	*35%*	*35%*	*36%*	*36%*	*37%*	*37%*	37%	37%	37%	37%
Tax	-2,109	-1,938	-2,177	-2,357	-2,474	-2,591	-2,796	-2,959	-3,145	-3,281	-3,702	-3,891	-3,930	-3,969
Tax rate	-18%	-18%	-18%	-18%	-18%	-18%	-18%	-18%	-18%	-18%	-18%	-18%	-18%	-18%
EBITDA post tax	**9,943**	**9,136**	**10,261**	**11,112**	**11,662**	**12,214**	**13,182**	**13,948**	**14,825**	**15,465**	**17,452**	**18,342**	**18,526**	**18,711**
Adjustments for contingent consideration liabilities paid	-1,493	-1,561	-1,528	-1,531	-1,073	-934	-883	-857	-853	-852	-962	-1,011	-1,021	-1,031
Working capital (Receivable, payable & Inventories)	9,670	10,318	11,014	11,822	11,618	11,954	12,548	13,179	13,818	14,305	16,142	16,966	17,135	17,307
as % of sales	29%	29%	29%	29%	28%	28%	28%	28%	28%	28%	28%	28%	28%	28%
Change in WC	115	-649	-696	-808	204	-336	-595	-630	-640	-486	-263	-168	-170	-171
Net Capex	-1,502	-1,606	-1,712	-1,828	-1,816	-1,867	-1,962	-2,061	-2,164	-2,240	-2,528	-2,657	-2,683	-2,710
as % of sales	-4%	-4%	-4%	-4%	-4%	-4%	-4%	-4%	-4%	-4%	-4%	-4%	-4%	-4%
Operating FCF	**7,064**	**5,320**	**6,326**	**6,946**	**8,977**	**9,077**	**9,742**	**10,399**	**11,169**	**11,886**	**13,699**	**14,507**	**14,652**	**14,798**
Discount Factor	100%	100%	93%	86%	79%	74%	68%	63%	58%	54%	37%	25%	23%	21%
NPV	**7,064**	**5,320**	**5,857**	**5,955**	**7,126**	**6,672**	**6,630**	**6,553**	**6,517**	**6,422**	**5,037**	**3,630**	**3,395**	**3,175**
Sum NPV (2023-2042)	112,507													
Perpetuity value	45,810	WACC	8%											
		Perpetuity												
Total EV	**158,316**	growth	1%											
Net debt	22,436													
Provisions	2,692													
Minorities	-421													
Market capitalisation	**133,609**													
Diluted Shares (m)	4,027													
Per share price (£)	**33.18**													

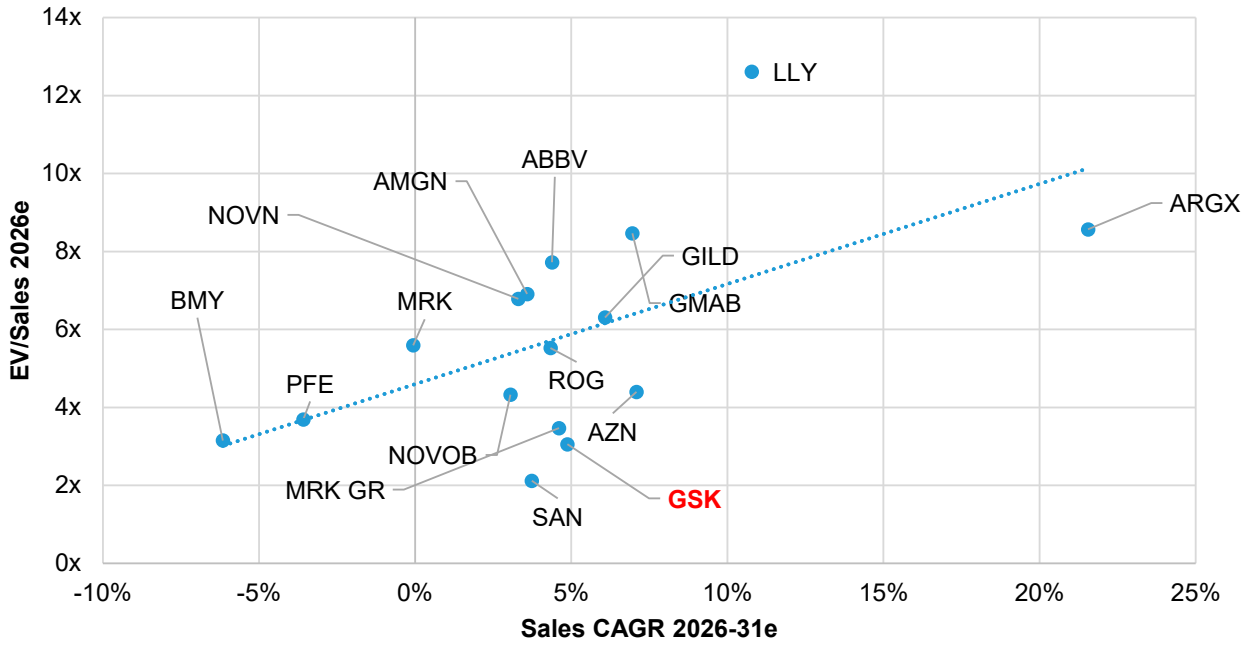
Perpetuity growth

WACC	**-1%**	**-2%**	**0%**	**1%**	**2%**
7%	36.01	34.60	37.82	40.24	43.63
8%	30.48	29.53	31.66	**33.18**	35.21
9%	26.14	25.49	26.93	27.92	29.20
10%	22.65	22.19	23.19	23.86	24.69
11%	19.79	19.47	20.17	20.63	21.20

Source: Company reports, Bernstein analysis and estimates

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EXHIBIT 16: **Global Pharma EV/Sales vs "26-31e sales CAGR regression**



Source: Company reports, Bernstein analysis and estimates

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EXHIBIT 17: **GSK summary of product sales estimates (in £m)**

	2025A	2026e	2027e	2028e	2029e	2030e	2031e	2032e	2033e	2034e	2035e	2036e
Tivicay	1,323	1,287	1,227	808	495	277	143	108	82	62	47	36
Triumeq	991	818	668	423	235	105	58	60	65	74	86	103
Juluca	656	667	654	434	300	134	56	43	34	27	21	16
Dovato	2,678	2,958	3,207	3,286	2,751	1,292	530	399	301	227	171	129
Rukobia (fostemsavir)	169	127	101	91	87	82	78	74	71	67	64	61
Long acting (treatment & prevention (PrEP))	1,841	2,386	2,738	3,342	4,519	3,862	4,785	5,172	5,609	6,139	6,649	7,183
HIV	**7,687**	**8,282**	**8,626**	**8,411**	**8,409**	**5,772**	**5,666**	**5,871**	**6,174**	**6,605**	**7,047**	**7,535**
Blenrep (BCMA; multiple myeloma)	17	179	331	475	641	908	1,253	1,658	2,118	2,591	2,976	3,257
Zejula	557	429	365	328	305	290	93	36	21	15	12	10
Jemperli (PD-1; cancer)	861	1,102	1,356	1,586	1,776	1,918	2,034	2,115	1,784	1,338	1,003	752
Ojjara (Sierra; myelofibrosis)	554	726	908	1,089	1,242	1,366	1,462	1,549	1,611	1,660	1,021	408
Jideytro (zidesamtinib; ROS1m NSCLC; Nuvalent)	0	8	33	112	223	391	648	989	1,373	1,758	2,092	2,344
Oncology	**1,977**	**2,445**	**2,992**	**3,591**	**4,188**	**4,874**	**5,489**	**6,346**	**6,907**	**7,361**	**7,103**	**6,772**
Hibsago (bepirovirsen)	0	0	185	392	611	879	1,072	1,266	1,361	1,387	1,412	1,456
Infectious Disease	**0**	**0**	**185**	**392**	**611**	**879**	**1,072**	**1,266**	**1,361**	**1,387**	**1,412**	**1,456**
Nucala	2,008	2,214	2,483	2,752	2,975	2,963	2,910	2,805	2,821	2,796	2,747	2,692
Exdensur	0	92	427	965	1,467	1,921	2,190	2,529	2,842	3,099	3,293	3,443
Other	29	40	40	40	40	40	40	40	40	40	40	40
Immuno-inflammation	**3,810**	**4,330**	**5,134**	**6,082**	**6,925**	**7,464**	**7,756**	**8,042**	**8,398**	**8,657**	**8,829**	**8,951**
Pandemic	**0**	**0**	**0**	**0**	**0**	**0**	**0**	**0**	**0**	**0**	**0**	**0**
Specialty Medicine	**13,474**	**15,057**	**16,938**	**18,477**	**20,132**	**18,989**	**19,983**	**21,526**	**22,840**	**24,010**	**24,392**	**24,714**
Arnuity Ellipta	36	36	36	36	36	36	36	36	36	36	36	36
Anoro Ellipta	542	541	551	559	563	422	316	237	178	133	100	75
Incruse Ellipta	119	101	91	83	77	57	43	32	24	18	14	10
Relvar/Breo Ellipta	1,017	923	859	762	678	606	491	399	326	267	219	181
Trelegy Ellipta	2,986	3,027	2,879	3,085	3,238	3,348	2,824	2,436	2,154	1,953	1,812	1,716
Respiratory	**7,068**	**6,807**	**6,442**	**6,412**	**6,355**	**6,121**	**5,261**	**4,599**	**4,093**	**3,706**	**3,410**	**3,185**
Dermatology	342	331	320	310	300	290	280	271	262	254	246	238
Augmentin	602	593	583	571	559	548	537	527	517	507	498	489
Blujepa (gepotidacin; antibiotic)	15	22	350	400	475	525	575	625	675	700	725	725
Avodart	340	336	331	327	323	318	314	310	306	302	298	294
Other General Medicines	**2,968**	**2,749**	**2,954**	**2,886**	**2,850**	**2,796**	**2,749**	**2,707**	**2,670**	**2,613**	**2,561**	**2,488**
General Medicines	**10,036**	**9,556**	**9,396**	**9,298**	**9,205**	**8,917**	**8,010**	**7,306**	**6,763**	**6,319**	**5,971**	**5,673**
Pharmaceuticals (marketed products)	**23,510**	**24,613**	**26,334**	**27,775**	**29,337**	**27,906**	**27,992**	**28,832**	**29,603**	**30,329**	**30,363**	**30,388**
Pharma Pipeline	**0**	**0**	**151**	**432**	**1,127**	**1,754**	**2,448**	**3,269**	**4,207**	**5,259**	**6,417**	**7,393**
Total Pharmaceuticals	**23,510**	**24,613**	**26,485**	**28,208**	**30,465**	**29,660**	**30,441**	**32,101**	**33,810**	**35,588**	**36,781**	**37,781**
Rotarix	546	542	564	589	617	648	682	720	763	810	863	921
Synflorix	159	144	131	120	110	102	95	89	83	79	75	71
Fluarix, FluLaval	303	262	229	206	190	181	176	171	167	163	158	154
Bexsero	1,150	1,224	1,309	1,377	1,442	1,507	1,572	1,635	1,700	1,760	1,822	1,886
Menveo	402	413	446	476	506	529	550	570	591	613	635	658
Boostrix	654	714	798	889	992	1,107	1,236	1,381	1,543	1,724	1,926	2,152
Infanrix, Pediarix	519	486	486	488	492	498	504	511	517	524	531	538
Hepatitis	643	633	629	627	628	631	638	647	660	677	697	721
Priorix, Priorix Tetra, Varilrix	425	358	382	408	436	465	497	531	567	606	647	691
Cervarix	23	7	2	1	0	0	0	0	0	0	0	0
Shingrix	3,558	3,670	3,456	3,603	3,751	3,892	4,027	4,158	4,286	4,405	4,484	4,569
Arexvy	593	558	700	700	700	700	700	700	700	700	700	700
Pandemic adjuvant +vaccine	0	0	0	0	0	0	0	0	0	0	0	0
Other	174	137	137	137	137	137	137	137	137	137	137	137
Vaccines	**9,149**	**9,148**	**9,269**	**9,622**	**10,003**	**10,398**	**10,814**	**11,250**	**11,714**	**12,196**	**12,675**	**13,199**
Vaccine Pipeline	8	77	425	550	700	850	800	850	900	950	1,000	1,000
Total Vaccine	**9,157**	**9,225**	**9,694**	**10,172**	**10,703**	**11,248**	**11,614**	**12,100**	**12,614**	**13,146**	**13,675**	**14,199**
Total sales	**32,667**	**33,828**	**36,179**	**38,566**	**41,167**	**40,907**	**42,055**	**44,201**	**46,424**	**48,734**	**50,456**	**51,980**

Source: Company reports, Bernstein analysis and estimates

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EXHIBIT 18: **GSK plc summary financial statements (in £m, except EPS & DPS in pence)**

	2025A	2026e	2027e	2028e	2029e	2030e	2031e	2032e	2033e	2034e	2035e	2040e
Income Statement (Core)												
Net revenues	32,667	33,828	36,179	38,566	41,167	40,907	42,055	44,201	46,424	48,734	50,456	56,937
Cost of products sold	-8,206	-8,200	-8,686	-9,313	-10,172	-9,648	-9,953	-10,405	-10,926	-11,412	-11,809	-13,354
Gross profit	**24,461**	**25,627**	**27,493**	**29,253**	**30,995**	**31,259**	**32,102**	**33,796**	**35,498**	**37,322**	**38,647**	**43,583**
SG&A	-8,989	-8,805	-9,515	-10,111	-10,877	-10,740	-10,830	-11,325	-11,936	-12,543	-12,962	-14,909
R&D	-6,568	-7,081	-7,416	-7,631	-8,088	-7,979	-8,107	-8,224	-8,452	-8,652	-8,856	-10,316
Royalty income	825	900	891	637	655	659	644	587	538	498	478	459
EBIT	**9,729**	**10,641**	**11,454**	**12,274**	**13,085**	**13,200**	**13,808**	**14,834**	**15,649**	**16,624**	**17,307**	**18,816**
Net financials	-508	-739	-967	-881	-768	-660	-517	-349	-161	14	80	420
Other non-operating income/(expense)	-10	-7	0	0	0	0	0	0	0	0	0	0
Pre tax earnings	**9,211**	**9,895**	**10,488**	**11,393**	**12,318**	**12,540**	**13,291**	**14,485**	**15,489**	**16,638**	**17,386**	**19,236**
Taxes	-1,584	-1,732	-1,835	-1,994	-2,156	-2,195	-2,326	-2,535	-2,711	-2,912	-3,043	-3,366
Net income	**7,627**	**8,163**	**8,652**	**9,399**	**10,162**	**10,346**	**10,965**	**11,950**	**12,778**	**13,726**	**14,344**	**15,870**
Net income to shareholders	**6,918**	**7,440**	**7,986**	**8,779**	**9,677**	**9,979**	**10,558**	**11,515**	**12,314**	**13,213**	**13,784**	**15,072**
EPS (diluted)	169.3	182.3	199.9	218.0	240.3	247.8	262.2	286.0	305.8	328.1	342.3	374.3
Dividend per share (proposed)	0.66	0.70	0.74	0.77	0.81	0.86	0.90	0.95	1.00	1.05	1.10	1.42
Balance Sheet												
Cash and equivalents	3,397	2,375	3,326	5,423	8,520	11,560	15,810	20,826	26,487	32,705	39,247	73,151
Accounts receivable	7,471	7,370	7,882	8,403	8,969	8,913	9,163	9,630	10,115	10,618	10,993	12,405
Inventories, net	5,924	5,907	6,257	6,708	7,327	6,950	7,169	7,495	7,870	8,221	8,506	9,620
Other current assets	709	709	709	709	709	709	709	709	709	709	709	709
Total current assets	**17,510**	**16,370**	**18,183**	**21,252**	**25,535**	**28,141**	**32,860**	**38,669**	**45,189**	**52,262**	**59,464**	**95,894**
PP&E	9,322	9,914	10,543	11,207	11,909	12,524	13,111	13,712	14,325	14,949	15,557	18,037
Right of use	726	726	726	726	726	726	726	726	726	726	726	726
Intangible assets & Goodwill	23,766	27,622	27,095	26,583	26,090	25,616	25,164	24,738	24,339	23,970	23,635	22,590
Other assets	9,794	9,886	9,978	10,070	10,162	10,254	10,346	10,438	10,530	10,622	10,714	11,174
Total assets	**61,118**	**64,518**	**66,525**	**69,838**	**74,422**	**77,260**	**82,207**	**88,283**	**95,109**	**102,529**	**110,096**	**148,420**
Short term debt	3,012	3,012	3,012	3,012	3,012	3,012	3,012	3,012	3,012	3,012	3,012	3,012
Accounts payable	15,381	15,978	16,925	18,147	19,821	18,799	19,394	20,275	21,289	22,237	23,011	26,021
Other current liabilities	2,998	2,734	2,620	2,520	2,453	2,406	2,392	2,378	2,366	2,357	2,352	2,327
Total current liabilities	**21,391**	**21,724**	**22,556**	**23,678**	**25,286**	**24,217**	**24,798**	**25,665**	**26,667**	**27,607**	**28,374**	**31,360**
Long term debt	14,708	21,808	21,808	21,808	21,808	21,808	21,808	21,808	21,808	21,808	21,808	21,808
Other long term liabilities	9,063	8,562	8,106	7,708	7,442	7,251	7,197	7,141	7,093	7,059	7,037	6,936
Total liabilities	**45,162**	**52,094**	**52,470**	**53,194**	**54,536**	**53,276**	**53,802**	**54,614**	**55,568**	**56,474**	**57,219**	**60,104**
Total stockholders' equity	15,956	12,424	14,055	16,644	19,885	23,984	28,405	33,669	39,541	46,055	52,877	88,316
Total liabilities and equity	**61,118**	**64,518**	**66,525**	**69,838**	**74,422**	**77,260**	**82,207**	**88,283**	**95,109**	**102,529**	**110,096**	**148,420**
Cash Flow												
Profit before tax	7,401	7,327	7,221	8,560	9,588	10,247	10,940	12,159	13,151	14,259	14,968	16,724
Depreciation, amortisation, impairment and other adjusting items	3,778	3,966	2,873	2,984	3,101	3,216	3,334	3,456	3,582	3,712	3,845	4,541
Change in working capital	-622	715	84	250	488	-587	125	88	155	94	112	82
Other adjustments	-1,614	-734	-581	-634	-750	-401	-404	-520	-684	-854	-919	-1,230
Taxation paid	-1,202	-1,280	-1,262	-1,496	-1,676	-1,791	-1,913	-2,126	-2,300	-2,493	-2,617	-2,925
Cash flows from operating activities	**7,741**	**9,995**	**8,335**	**9,664**	**10,751**	**10,684**	**12,083**	**13,057**	**13,905**	**14,718**	**15,388**	**17,192**
Net Acquisitions	-1,719	-7,100	0	0	0	0	0	0	0	0	0	0
Net Capex	-1,324	-1,502	-1,606	-1,712	-1,828	-1,816	-1,867	-1,962	-2,061	-2,164	-2,240	-2,528
Others, net	-1,190	-4,726	-1,296	-1,351	-1,408	-1,468	-1,530	-1,595	-1,662	-1,732	-1,804	-2,214
Cash flows from investing activities	**-4,233**	**-13,328**	**-2,902**	**-3,063**	**-3,236**	**-3,284**	**-3,397**	**-3,557**	**-3,723**	**-3,895**	**-4,044**	**-4,742**
Cash flows from financing activities	**-3,685**	**2,311**	**-4,482**	**-4,504**	**-4,417**	**-4,360**	**-4,437**	**-4,484**	**-4,521**	**-4,604**	**-4,803**	**-5,973**
Net change in cash/cash equivalents	**-196**	**-1,022**	**951**	**2,097**	**3,097**	**3,040**	**4,249**	**5,016**	**5,661**	**6,219**	**6,541**	**6,477**
Cash at the beginning of year	3,403	3,207	2,185	3,136	5,233	8,330	11,370	15,620	20,636	26,297	32,515	66,484
Cash and bank overdrafts at the end of the year	3,207	2,185	3,136	5,233	8,330	11,370	15,620	20,636	26,297	32,515	39,057	72,961
Cash at the end of year	**3,397**	**2,375**	**3,326**	**5,423**	**8,520**	**11,560**	**15,810**	**20,826**	**26,487**	**32,705**	**39,247**	**73,151**

Source: Company reports, Bernstein analysis and estimates

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Justin Smith +44 20 7762 5899 justin.smith@bernsteinsg.com

EXHIBIT 19: **GSK Pipeline Catalyst Calendar**

Date	Stock	Milestone	Comment	Source:
18/09/2026	GSK LN	ROS1 TKI (zidesamtinib) 2L FDA approval deadline	Vast majority of value creation will be in 1L due to 7 + years duration of therapy. FDA submission for 1L segment in 2H26. But 2L approval should increase conviction in rationale of $11bn Nuvalent acquisition.	9/6/26 GSK IR deck slide 21
26/10/2026	GSK LN	bepirovirsen US approval (PDUFA) deadline	Would be the first FDA approved treatment to achieve "Functional Cure" in hepatitis B ($3bn market)	9/6/26 GSK IR deck slide 21
27/11/2026	GSK LN	ALK TKI (neladalkib) 2L FDA approval deadline	2L+ setting (vast majority of value creation will be in 1L due to 7 + years duration of therapy. 1L phase 3 (ALKAZAR) ongoing. But 2L approval should increase conviction in rationale of $11bn Nuvalent acquisition.	9/6/26 GSK IR deck slide 21
2H27	GSK LN	Phase 3 (ARROS-1) read out for NUVL Jideytro (ROS1 inhibitor) for non-small cell lung cancer first line	1L+ setting (vast majority of value creation will be in 1L due to 7 + years duration of therapy).	9/6/26 GSK IR deck slide 21
2H27	GSK LN	Phase 3 readout for Cabenuva 3X yearly (HIV Treatment)	3X yearly injections aligns with frequency of regular health check ups for patients with HIV	9/6/26 GSK CT IR deck slide 43
2027	GSK LN	Phase 3 read out for Exdensur (nasal polyps)	2X yearly dosing (Exdensur) would be the longest acting biologic on approved for nasal polyps	9/6/26 GSK CT IR deck slide 19-22
2028	GSK LN	Phase 3 read out efimosfermin (liver disease; MASH)	Potential best-in-class FGF21 for Steatotic Liver Disease ($15bn market)	9/6/26 GSK CT IR deck slide 17&18
2028	GSK LN	Phase 3 read out velztanib (oncology; GIST)	Next generation tyrosine kinase inhibitor that inhibits all primary and key secondary KIT mutations	9/6/26 GSK CT IR deck slide 39
2028	GSK LN	Phase 3 read out Blenrep treatment naïve (DREAMM 10)	Additional upside potential from use in earlier treatment lines due to duration of therapy	9/6/26 GSK CT IR deck slide 30
2028	GSK LN	Phase 3 read out risvutatug (ADC) small cell lung cancer	Potential to be an "oncology pipeline in a single asset" - first of 8 potential indications	9/6/26 GSK CT IR deck slide 38
2029	GSK LN	Phase 3 read out NUVL ALK inhibitor (neladalkib) first line	1L+ setting (vast majority of value creation will be in 1L due to 7 + years duration of therapy).	9/6/26 GSK IR deck slide 21
2030	GSK LN	Phase 3 read out VH184 (2X yearly injectable) in HIV Treatment	First 2X yearly injection for HIV Treatment	9/6/26 GSK CT IR deck slide 45

Source: Company reports, Bernstein analysis and estimates

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BERNSTEIN TICKER TABLE

			29 Oct 2026		TTM	Adjusted EPS				Adjusted P/E (x)	
Ticker	Rating	Cur	Closing Price	Price Target	Rel. Perf.	Cur	2025A	2026E	2027E	2025A	2026E	2027E
GSK.LN (GSK)	O	GBp	1,814.00	**3,004.00**	(9.8)%	GBP	1.69	**1.82**	**2.00**	10.7	10.0	9.1
OLD				3,010.00				1.79	2.06			
EDME			1,603.66									

PRICE TARGET CHANGE / ESTIMATE CHANGE IN BOLD

O - Outperform, M - Market-Perform, U - Underperform, NR - Not Rated, CS - Coverage Suspended

Source: Bloomberg, Bernstein estimates and analysis.

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VALUATION METHODOLOGY

GSK plc

Our 3,004p price target is the simple average of 2027-29e EV/EBITA multiples and a DCF (WACC 8%, terminal growth 1%). For the former we value GSK at a 0% premium to large cap pharma peers on account of the dilution from the NUVL acquisition.

RISKS

GSK plc

Downside risk: (1) If the late-stage pharma pipeline fails, in which case our 2031 sales estimate would drop to £40bn (GSK’s guidance is >£40bn) (2) If core EBITA margins don’t expand between 2025 and 2030 (3) If the global vaccines business doesn’t grow until 2034.

RATINGS DEFINITIONS, BENCHMARKS AND DISTRIBUTION

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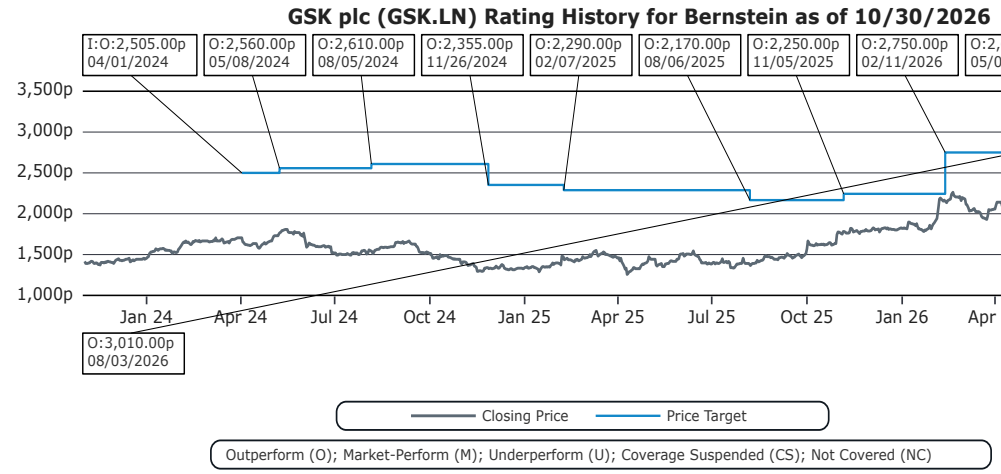
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Market-Perform (Bernstein Brand) Neutral (Autonomous Brand)	HOLD	35.6%	16.1%
Underperform	SELL	12.1%	13.6%

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